

FROM 1900 TO 0700, ORGANIZED. NOTHING LEFT IN YOUR HEAD AT SHIFT CHANGE.

My intention for this shift (one word or phrase): _____

Labs pending: ___ Imaging: ___ Consults awaiting: ___ Orders to clarify: ___

Still outstanding at 0645 (must hand off verbally): ____

SBAR Handoff Block (One Copy Per Patient — Photocopy or Reprint)

Fill one block per patient starting at 0630. The gut-feeling line is not optional — say it out loud in report even if it sounds soft. Oncoming nurses act on gut feelings.

SITUATION — Patient initials: ___ Bed: ___ Primary Dx: ___ Reason admitted: ___

BACKGROUND — Relevant history: ___ Allergies: ___ Code status: ___ Active drips and current rates: ___

ASSESSMENT — Vitals stable: Y / N Pain: ___/10 Neuro status: ___ Respiratory: ___ Skin / wound: ___ Last voided or output: ___ Significant change this shift: ___

RECOMMENDATION — What the oncoming nurse must specifically watch for: ___

Pending items to hand off (labs, imaging results, orders, family calls): ___

My gut feeling about this patient right now: ___

End-of-Shift Checklist (Hard Stop at 0645)

Two minutes before you give report. Do not skip this because you feel rushed. Skipping it is why things get missed.

- ☐ All medications administered and documented — no gaps
- ☐ Critical values reported, documented, and loop closed or explicitly handed off
- ☐ Pending orders, awaited labs, and imaging results communicated verbally to oncoming nurse
- ☐ Family questions or outstanding callbacks named in report
- ☐ Nothing left unsigned or uncharted
- ☐ IV lines, drips, and pump rates visually confirmed before leaving room
- ☐ Personal items accounted for: badge, stethoscope, keys, phone charger, lunch bag

One thing I did well this shift: _____

One thing I would change if I ran it again: _____

Something to follow up on next time I work this unit: _____

Shift stress level right now (1 to 10): ___

Post-Shift Wind-Down Protocol

Your nervous system does not automatically leave code-blue mode when you clock out. Cortisol does not read your schedule. This protocol is not optional self-care language — it is physiological maintenance for a job that runs on human performance.

- ☐ Drive home with no work calls and no shop-talk podcast — window down or music only, nothing that keeps the clinical brain firing
- ☐ Change out of scrubs immediately when you walk in the door — do not sit on your bed in them
- ☐ Eat something light if hungry — large heavy meals delay sleep onset and fragment what you do get
- ☐ Write any lingering work thoughts on paper before lying down — second brain dump, clears the loop
- ☐ Blackout curtains closed, white noise or fan on, phone set to Do Not Disturb with emergency contacts whitelisted
- ☐ Set one alarm only — protect a minimum of seven hours, eight if the shift was hard
- ☐ Confirm your next shift date and time right now so you do not surface from sleep unsure if you are late

Next shift: Day ___ / Date ___ / Report time ___

Sleep target: down by ___ up by ___